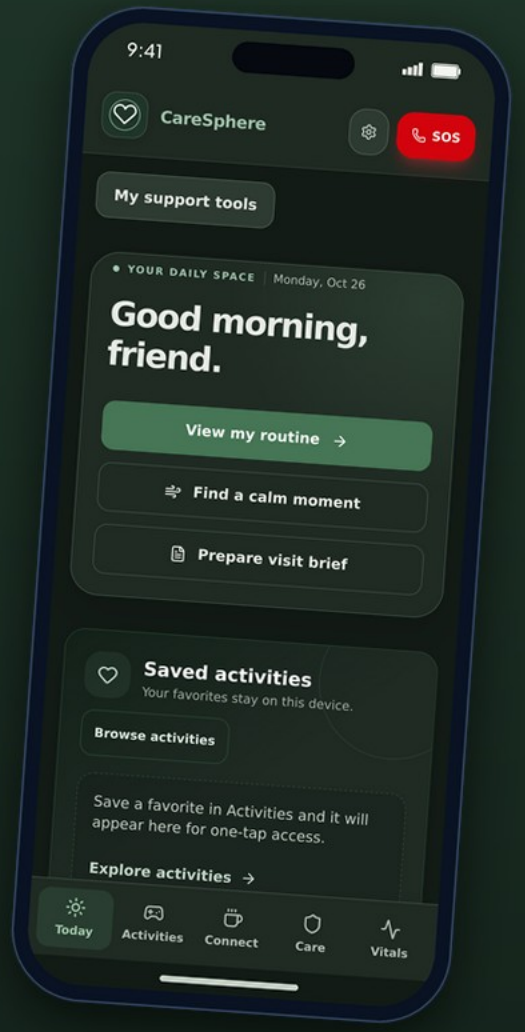
CARESPHERE · BUSINESS & VALIDATION APPENDIX

Supplemental Deck

A clear opportunity. An honest starting point.

Market sizing · Competitors · Go-to-market Business model · Risks · Validation roadmap

Arnav Kochar · October 26, 2026



EXECUTIVE SNAPSHOT

02

Daily support, without a care-data feed.

A local-first companion—not a replacement for clinicians or care teams.

THE NEED

Less fragmentation

Routines, calming activities, notes and visit preparation can be scattered across different tools.

THE PRODUCT

Choice and control

Bring useful daily tools into one adjustable experience. Core features do not require an account.

THE NEXT TEST

Prove usability

Start with voluntary caregiver-led pilots. Validate accessibility, repeat use and willingness to support the service.

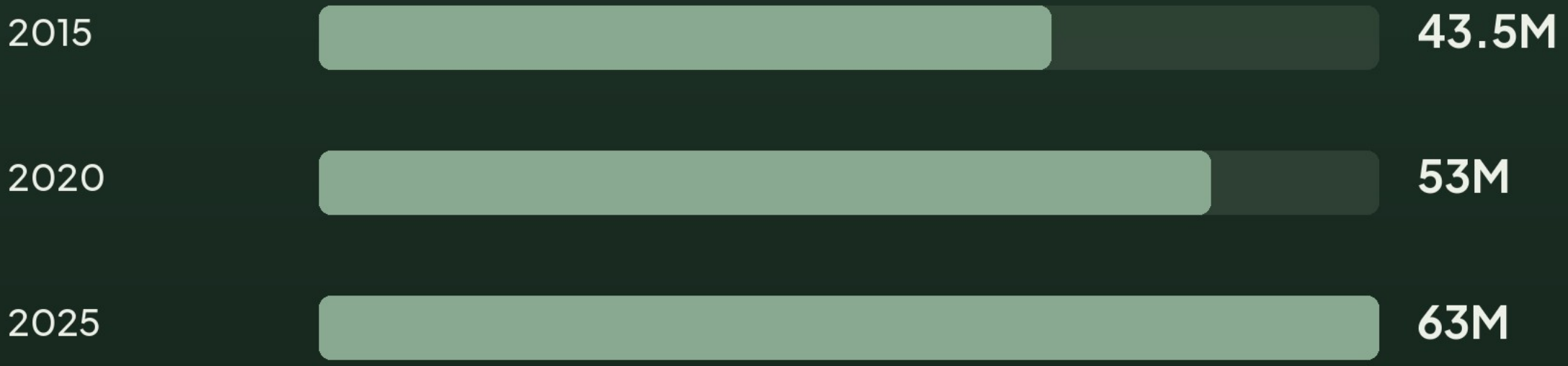
Sources: P1

MARKET CONTEXT

03

Family caregiving is already widespread.

U.S. family caregivers of adults or children with medical conditions or disabilities.



Published survey estimates—not app users, purchasers, or a forecast. Do not add overlapping audiences.

Sources: M1

CUSTOMERS AND USERS

04

Start narrow. Keep the broader mission.

The beneficiary, the person choosing the app, and a possible sponsor may differ.

INITIAL USER HYPOTHESIS

Caregiver-led use

Adults supporting an older family member who wants simple routines, notes and visit preparation.

PEOPLE WE DESIGN FOR

Different needs

Older adults, people living with dementia and autistic people. Do not assume every person needs a caregiver.

POTENTIAL SUPPORTER

Community sponsors

Aging services and community organizations could support onboarding or access. No partnerships or sales are claimed.

Sources: P1 | Segments and buyer roles are hypotheses

TAM · SAM · SOM

05

A practical starting point for market sizing.

U.S. caregiver participants; TAM is an audience ceiling, not demonstrated demand.

TAM · TOTAL ADDRESSABLE MARKET

59M

U.S. caregivers supporting adults, 2025. A broad starting population—not all addressable or likely to adopt.

SAM · SERVICEABLE AVAILABLE MARKET

2.95M

59M × an assumed 5% fit for language, access, needs and willingness. This filter has not been measured.

SOM · SERVICEABLE OBTAINABLE MARKET

120

12 potential partners × 100 invites × 25% activation × 40% 90-day retention. A 24-month target, not traction.

Sources: M2 | SAM/SOM hypothetical; cards not to scale

MARKET SENSITIVITY

06

The serviceable share is still an assumption.

Change the assumed fit—not the published 59M caregiver baseline.



These are arithmetic scenarios, not research findings or confidence intervals.

Sources: M2 | No measured eligibility or adoption rate

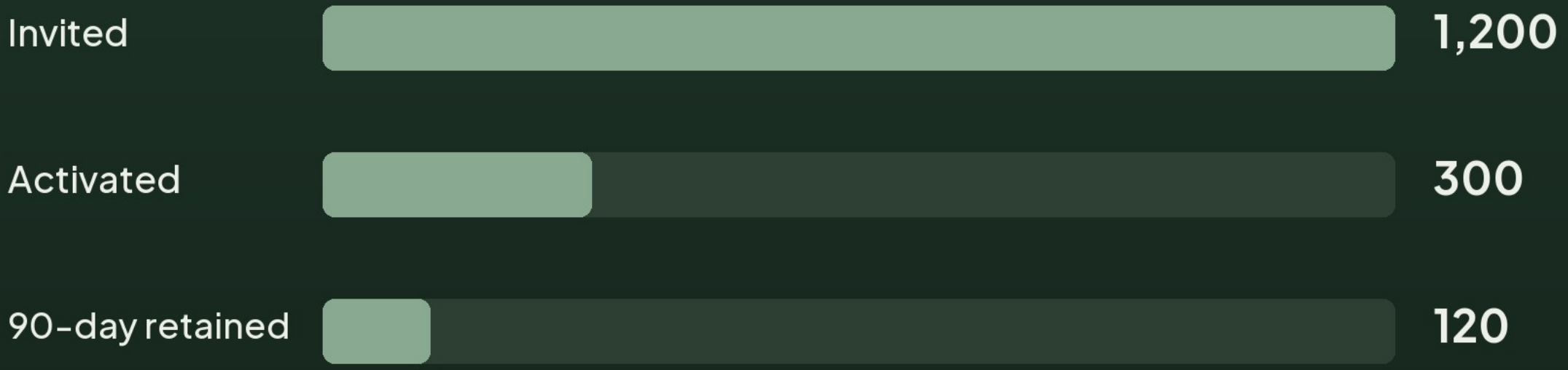
CareSphere AI

BOTTOM-UP OBTAINABLE MARKET

07

Build the obtainable market bottom-up.

Illustrative 24-month outreach model; every conversion rate requires testing.



12 prospective partners × 100 unique invitations. No partner agreements or invitations are claimed.

CareSphere planning appendix · October 26, 2026 | 25% activation; 40% retention of activated

COMPETITOR COMPARISON

08

Different tools solve different jobs.

A comparison of documented focus—not an all-or-nothing feature scorecard.

Dimension	CareSphere	Medisafe	CaringBridge	Lotsa Helping Hands
Main job	Daily support	Medication routines	Health updates	Volunteer help
Examples	Routines, calming, visit prep	Dose reminders; Medfriend	Journal updates; support requests	Help calendar; task sign-ups
Sharing model	Review exports; Jitsi calls	Missed-dose alerts to Medfriend	Private/public health pages	Shared community coordination
CareSphere lens	Personal daily use	Medication-focused alternative	Communication alternative	Task-coordination alternative

Sources: P1, C1, C2, C3

POSITIONING

09

Compete on calm, not feature count.

A differentiation hypothesis that must survive user testing.

POTENTIAL ADVANTAGE

A coherent routine

Activities, comfort settings, notes and visit preparation in one local-first experience. No claim of uniqueness.

REAL TRADE-OFF

Less automatic coordination

No invisible sharing or care-team feed. People who need shared task dispatch may prefer a coordination tool.

WHAT COULD ENDURE

Trust earned over time

Accessible design, reliable reminders and understandable privacy. Code alone is not a defensible moat.

Sources: P1, C1, C2, C3 | Positioning is a hypothesis, not a benchmark

Learn through trusted, small-scale channels.

Proposed outreach—not signed distribution or institutional endorsements.

DISCOVER

Listen first

Interview adult caregivers and independent users. Explore accessibility barriers and current workarounds; do not seek diagnoses.

PILOT

Supported onboarding

Invite opt-in cohorts through interested community organizations. Use fictional care data in demonstrations.

EXPAND ONLY IF USEFUL

Earn referrals

Offer understandable setup guides and accessible walkthroughs. Expand channels after repeat use and safe operation are demonstrated.

SUSTAINABILITY MODEL

Keep essential support accessible.

A model to validate—not a launched subscription or promised revenue.

CORE EXPERIENCE

Free-first

Protect account-free daily tools. Do not sell care data or make an ad-targeting feed the business model.

POSSIBLE FUNDING

Sponsored support

Test grants or organizational sponsorship for onboarding, training and maintenance—not access to private records.

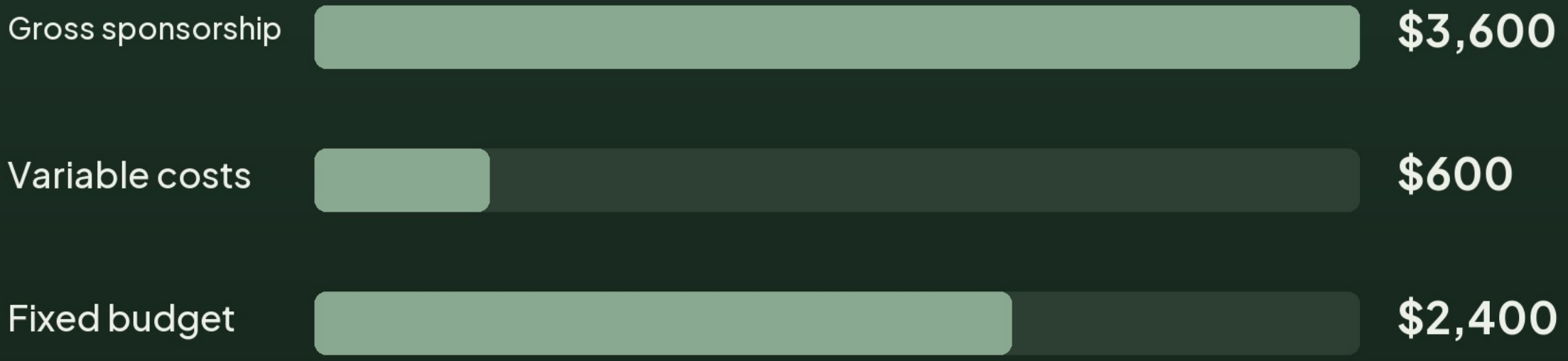
DECISION GATE

Validate willingness to pay

Interview potential supporters, cost the service, and clarify obligations before offering contracts. Payment flows are not demonstrated.

A transparent budget—not a profit forecast.

One hypothetical annual cohort: 100 sponsored seats at $36 per seat per year.



$600 remains before labor, tax, payment fees, legal/compliance work and contingencies.

CareSphere planning appendix · October 26, 2026 | $6 variable/seat/year; $2,400 fixed/year

Local-first has clear boundaries.

Web, iPhone and Android source exist; native-device validation remains pending.

ON THE DEVICE

Personal care tools

Account-free core and encrypted local care data. Selected records can be reviewed before export. A lost key or device can mean lost access.

OUTSIDE THE DEVICE

Explicit external services

Jitsi video calls use external infrastructure. Apple's optional on-device model needs eligible hardware, OS and model availability.

NEVER IMPLY

Clinical or emergency coverage

No diagnosis, verified-dose guarantee, continuous monitoring or emergency dispatch. Reference links and visit summaries support information organization.

Sources: P1

Built is not the same as validated.

Research informs design; it does not validate CareSphere's clinical outcomes.

SOURCE AND ARTIFACTS

What can be inspected

Feature source across three platforms; real mobile-web screenshots; editable decks and a working-web demonstration workflow.

RESEARCH RATIONALE

What papers suggest

Digital support, individual needs and meaningful video interaction inform design. These studies did not test CareSphere.

NOT ESTABLISHED

What still needs evidence

Verified adoption, retention, revenue, clinical outcomes, signed partners, native store releases and physical-device reliability.

Sources: P1, R1, R2, R3

Advance when the evidence is ready.

Relative planning windows—not fixed release dates or completed tests.

0–3 MONTHS · PROPOSED

Accessibility gate

Review VoiceOver, Dynamic Type, keyboard navigation, contrast and language flows with consenting users.

3–6 MONTHS · PROPOSED

Reliability gate

Test reminders across time zones and restarts; BLE disconnects; encryption, export and recovery on real devices.

6–12 MONTHS · PROPOSED

Pilot gate

Run a small opt-in cohort only after critical safety issues are resolved. Measure chosen-task success and repeat usefulness before widening access.

Trust risks deserve an explicit plan.

No mitigation below should be mistaken for a completed audit.

Reliability	Missed reminders or stale readings	Device/time-zone testing; visible failure states
Privacy	Sensitive exports or shared rooms	Review-before-share; explain external services
Accessibility	People cannot use core tasks	User-led testing; assistive-tech review
Adoption	Broad mission, weak repeat use	Narrow pilot; measure meaningful return use
Sustainability	Support costs exceed funding	Cost labor; test sponsorship before scaling

Measure useful support—not screen time.

Metrics below are proposed definitions and targets, not achieved results.

TASK SUCCESS

Can people do what they need?

Observe routine entry, finding a calming tool, and reviewing a visit brief. Record accessibility barriers with consent.

ACTIVATION TARGET

25% of invitees

Complete one personally chosen useful action within seven days. This is an input to the SOM scenario, not measured conversion.

RETENTION TARGET

40% of activated

A chosen useful action during days 61–90. Measure with opt-in follow-up; no hidden collection of care records.

Help shape CareSphere’s next chapter.

Seeking feedback and potential pilot collaborators—not claiming endorsements.

Three things to learn next

Can people use core tasks comfortably?

Does the app remain useful after onboarding?

Can support be funded without selling care data?



Explore the GitHub project

github.com/engineer2025-sudo/CareSphere

The evidence trail stays visible.

Published estimates and design research are distinct from the model assumptions.

M1 · AARP / NAC, Caregiving in the US 2025
https://www.aarp.org/pri/topics/ltss/family-caregiving/caregiving-in-the-us-2025/

M2 · AARP, July 24, 2025: 59M caring for adults; 63M total
https://www.aarp.org/press/releases/2025-07-24-new-report-reveals-crisis-point-for-americas-63-million-family-caregivers.html

M3 · U.S. Census, Vintage 2024: 61.2M Americans age 65+
https://www.census.gov/newsroom/press-releases/2025/older-adults-outnumber-children.html

R1 · Ditton et al. (2025), Dementia
https://doi.org/10.1177/14713012241302371

R2 · Maun et al. (2024), Journal of Autism and Developmental Disorders
https://doi.org/10.1007/s10803-023-06015-5

R3 · Noh et al. (2025), Geriatric Nursing
https://doi.org/10.1016/j.gerinurse.2025.03.042

Sources: M1, M2, M3, R1, R2, R3

Audit the claims. Change the inputs.

No third-party market forecast, competitor score, or invented traction is needed.

C1 · Medisafe, official app features
https://medisafe.com/download-the-app

C2 · CaringBridge, official platform overview
https://www.caringbridge.org/

C3 · Lotsa Helping Hands, official overview and care calendar
https://lotsahelpinghands.com/

P1 · CareSphere app source inspected at 0af7c225; build/device checks pending
https://github.com/engineer2025-sudo/Care/tree/0af7c22518a7fa7d8caa3f795c9d0b022f7373a8

Unvalidated inputs: 5% serviceable fit; 12 partners; 100 invites each; 25% activation; 40% retention; $36/seat/year; $6 variable/seat/year; $2,400 fixed/year. No willingness-to-pay or acquisition-cost data supplied.

Sources: C1, C2, C3, P1